

Mapping worksheet

Fill this in before writing code. Every infusion device carries the same facts in different words. This page is where your words meet OpenBedside's. When it is complete, the adapter is mostly copying it into two tables and one function.

Device: _____ Firmware / protocol version: _____

1. Transport

Question	Your answer
How does data leave the device? (serial, TCP, UDP, vendor server API, file)	
Who starts the connection, the device or the gateway?	
Message framing (one JSON per line, length prefix, HL7, binary)	
How often does the device report while infusing? While idle?	
Does it send on change, on a timer, or only when asked?	
What happens when the connection drops? Does it buffer and resend?	
Authentication or encryption on the link	

2. Identity and containment

OpenBedside field	Meaning	Your field / how derived
<code>device_id</code>	stable, unique per physical device (EUI-64 or serial). The hospital registers the device by vendor plus this id	
<code>vendor</code> , <code>model</code>	shown on the status page and in HL7; <code>vendor</code> must be the same string every time	

OpenBedside field	Meaning	Your field / how derived
reported_patient_id	a patient identifier the device itself holds (a wristband scanned at the pump), or None. Never looked up or guessed by the adapter	
identifier the nurse scans	what is on the label the hospital scans to tie the device to a patient: serial, asset tag, EUI-64?	
module_id	stable per pump module or channel	
modules per device	a two-channel pump is ONE device with TWO modules	
device_clock	the device's own time, if it reports it	

3. States

Your state word	Meaning on your device	OpenBedside <code>PumpStatus</code>
	infusing at the programmed rate	INFUSING
	keep vein open after VTBI reached	KVO (still infusing)
	paused or on hold by a clinician	PAUSED
	stopped, no program	IDLE
	program finished	COMPLETE
	any alarm state	ALARM

Anything not listed maps to `UNKNOWN`, and the checker warns about it.

4. Values and units

OpenBedside field	Unit(s) allowed	Your field	Your unit string
actual rate (what the channel is doing now)	mL/h		

OpenBedside field	Unit(s) allowed	Your field	Your unit string
programmed rate, per source	mL/h		
VTBI	mL		
volume delivered	mL		
volume remaining	mL		
drug name	text		
concentration	mg/mL, [iU]/mL, ...		
dose rate	ug/kg/min, [iU]/h, ...		
patient weight	kg		

Rules: a number never travels without its unit; unknown is left empty; a dose rate is only useful with the concentration (and weight, if weight based) that produced it.

5. Primary and secondary

Question	Your answer
How does the device show a piggyback (secondary) is programmed?	
While the secondary runs, which rate does it report as "current"?	
Does the primary resume automatically when the secondary finishes?	

6. Things OpenBedside will infer, and whether your device tells you directly

Event	Device reports it? (message / field)
delivery start	
delivery stop	
delivery complete	

Event	Device reports it? (message / field)
rate change	
KVO start	
secondary to primary	

OpenBedside infers all of these from snapshots and labels them *derived*. If your device reports them directly, note it: a future release can pass device-reported events through as such.

7. What your device does NOT send

List anything a clinician would expect in the record that your device cannot supply (for example: drug name, concentration, weight, alarms). Missing data is a finding, not a detail. It goes in your integration notes to the hospital.